This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 30 SEPTEMBER 2026

Patient Group Direction

for the supply of Amoxicillin or Metronidazole for the treatment of Acute Dental Infection, as a bridging antibiotic

Acute Dental Infection, as a bridging antibiotic

Adults aged 18 years and over.

Patient Group Direction, version 010, issued 30 September 2026. Amoxicillin and metronidazole, adults 18 and over, spreading or systemic dental infection only.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

The supply of a short course of antibiotic to an adult with an acute dental infection that is spreading or causing systemic upset, to bridge the time until they can be seen by a dentist, by a registered pharmacist or registered pharmacy technician in a community pharmacy, without a prescription.

What it authorises

- Amoxicillin 500mg capsules, first line, in adults aged 18 years and over.
- Metronidazole 200mg tablets, in adults aged 18 years and over who are penicillin-allergic.

What it does not cover

- Toothache or a localised dental abscess without spreading infection or systemic features. National guidance is that these do not need an antibiotic; they need a dentist.
- Facial swelling that is closing the eye, affecting swallowing or breathing, or spreading to the neck, or a patient who is systemically very unwell. Refer as an emergency.
- Pain relief. This PGD supplies no analgesia.
- Anyone under 18.

Summary of SDCEP and NICE CKS guidance for acute dental infection

Scottish Dental Clinical Effectiveness Programme, Drug Prescribing for Dentistry, dental abscess; NICE Clinical Knowledge Summaries, Dental abscess; NHS primary care antimicrobial prescribing guidance. Summarised 9 September 2026.

Overview

An acute dental infection begins in the pulp or the periodontal tissues and is contained, at first, by the immune response and by the anatomy of the tooth.

The treatment is DRAINAGE and removal of the cause: opening the tooth, extraction, or drainage of a swelling. That is dental treatment, and a pharmacy cannot provide it.

- An antibiotic does not drain an abscess and does not remove the cause.
- A walled-off abscess has a poor blood supply, so very little antibiotic reaches the infection.

When an antibiotic IS indicated

SDCEP: antibiotics are required only in cases of SPREADING INFECTION or SYSTEMIC INVOLVEMENT.

NHS and NICE CKS guidance is to the same effect: antibiotics are not generally indicated for otherwise healthy people with no signs of spreading infection.

- Spreading: facial swelling, cellulitis, lymph node involvement.
- Systemic: fever, malaise, rigors. SDCEP takes a temperature above 38C as indicating systemic involvement. In this PGD a rigor is treated as a sepsis red flag and refers (Appendix 1); it is not a reason to supply.
- CKS also supports treating high-risk individuals, such as the immunocompromised or those with poorly controlled diabetes, to reduce the risk of complications.

When an antibiotic is NOT indicated

A localised infection in an otherwise healthy person: pain and tenderness at one tooth, with or without a small amount of adjacent gum swelling, and no fever, no lymphadenopathy and no facial swelling.

For these patients the mainstay is DENTAL TREATMENT plus ANALGESIA. CKS advises ibuprofen first line, paracetamol where ibuprofen is unsuitable.

Supplying an antibiotic here does not help the patient and contributes to antimicrobial resistance.

What is an emergency

Airway compromise, difficulty swallowing or breathing, drooling, or a change in the voice.

Swelling of the floor of the mouth or a raised tongue, suggesting Ludwig angina.

Trismus, periorbital or orbital involvement, rapidly spreading swelling, or signs of sepsis.

These need 999 or same-day emergency care, not an antibiotic and not a dental appointment.

How this PGD applies that guidance

This PGD supplies a BRIDGING antibiotic to the spreading or systemic group only, while they wait for urgent dental assessment, and refers the emergency group immediately.

It deliberately does NOT supply to the localised group, and says so in terms, because that is what the guidance requires.

The Patient Group Direction

How to use this PGD

The three outcomes

Work out which of these applies before you think about a medicine at all.

1. LOCALISED INFECTION ONLY, otherwise healthy patient: NO ANTIBIOTIC. Analgesia advice and an urgent dental appointment. This is most patients who present, and it is the correct answer for them.

2. SPREADING OR SYSTEMIC INFECTION, no emergency red flag: ANTIBIOTIC UNDER THIS PGD, and an urgent dental appointment as well. This is the bridging cohort.

3. ANY EMERGENCY RED FLAG from Appendix 1: 999 or same-day emergency care. No antibiotic here, and no delay to arrange one.

Outcomes 1 and 3 are the majority. A pharmacy that supplies an antibiotic to most people who walk in with toothache is getting this wrong.

On the swelling question, which caused the error

That instruction was right, and it is kept, but it was applied too widely. Swelling that threatens the airway, closes the eye, limits mouth opening or is spreading rapidly is an EMERGENCY, and Appendix 1 sends it to 999 or same-day care. Localised facial swelling without those features is not an emergency: it is a sign of spreading infection, and it is one of the reasons to supply an antibiotic while the patient waits for a dentist.

A note on the penicillin-allergy arm

This PGD uses metronidazole for penicillin-allergic patients, following SDCEP, which gives it as an alternative in dental infection. NICE CKS and several NHS antimicrobial formularies instead give clarithromycin as the penicillin-allergy alternative for dental abscess, with metronidazole used as an adjunct. Both positions exist in current UK guidance. This document follows SDCEP and states the divergence here rather than leaving a pharmacist to discover it. It should be revisited at the next review.

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Patient Group Direction

for the supply, for the treatment of Acute Dental Infection, as a bridging antibiotic, of:

Amoxicillin, first line

Patient Group Direction for the supply of amoxicillin 500mg capsules as a bridging antibiotic in acute dental infection with spreading or systemic involvement, in adults aged 18 years and over who are not penicillin-allergic.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- Must have completed training relevant to acute dental infection, documented and overseen by the Get Real Health team.
- Must be able to distinguish a LOCALISED dental infection from a SPREADING or SYSTEMIC one, because that distinction decides whether an antibiotic is indicated at all. This is the single most important competence for this PGD.
- Must be able to recognise every emergency red flag in Appendix 1, in particular airway compromise, difficulty swallowing, trismus and periorbital involvement, and must act on them the same day rather than supplying.
- Must be able to explain to a patient with a localised infection why an antibiotic is not being supplied, and why that is the right answer rather than a refusal of care.
- Must be able to take and record a penicillin allergy history in the patient own words, distinguishing true allergy from intolerance.
- Must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005. This PGD covers adults only.

The PGD

Indication	Bridging antibiotic care for an acute dental infection with signs of spreading infection or systemic involvement, in an adult awaiting urgent dental assessment. First-line agent. This is not definitive treatment: the infection is treated by the dentist.
The three outcomes	**Decide which of these applies BEFORE considering any medicine.**

	1. LOCALISED INFECTION ONLY, otherwise healthy patient: NO ANTIBIOTIC. Analgesia advice and an urgent dental appointment. This is most patients who present, and it is the correct answer for them. **2. SPREADING OR SYSTEMIC INFECTION, no emergency red flag: ANTIBIOTIC UNDER THIS PGD, and an urgent dental appointment as well. This is the bridging cohort.** **3. ANY EMERGENCY RED FLAG from Appendix 1: 999 or same-day emergency care. No antibiotic here, and no delay to arrange one.** **An antibiotic under this PGD is a bridge to dental care, not a treatment. It does not drain the infection and it does not remove the cause. The dental appointment is the treatment, and it is needed whether or not an antibiotic is supplied.**
Inclusion criteria	● Adults aged 18 years and over. ● An acute dental infection with a presumed bacterial source in a tooth or its supporting tissues. ● **AT LEAST ONE sign of spreading infection or systemic involvement, from the list below.** ● Temperature 38C or above. ● Regional lymphadenopathy: tender, enlarged nodes in the neck or under the jaw. ● Facial swelling, or swelling extending beyond the tooth and its immediate gum, that does NOT meet any emergency red flag in Appendix 1. ● Malaise, feeling generally unwell. RIGORS ARE NOT ON THIS LIST: a rigor is a sepsis red flag in Appendix 1 and refers. ● Cellulitis: diffuse redness and swelling spreading into the soft tissues of the face, without red flags. ANY SPREAD TO THE NECK is an Appendix 1 emergency, not a reason to supply. ● **OR the patient is at higher risk of complications from a dental infection even where it appears localised: significant immunosuppression, or poorly controlled diabetes. Record which applies and why you judged the risk higher.** ● **No emergency red flag from Appendix 1 present.** ● Unable to obtain definitive dental treatment before the infection would be expected to worsen, and willing and able to arrange an urgent dental appointment within 24 to 48 hours. ● NOT penicillin-allergic. Where the patient is, use the metronidazole arm. ● Capable of giving valid informed consent, and consent given. ● No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following is present.** ● **ANY emergency red flag from Appendix 1. These are 999 or same-day emergency presentations, not dental appointments.** ● **A LOCALISED infection with no sign of spread and no systemic feature, in a patient who is not at higher risk. An antibiotic is not indicated. Give analgesia advice and arrange urgent dental care. This is not a failure of the service; it is the service working correctly.** ● Under 18 years of age. Children with dental infection are referred: this PGD carries no paediatric dose, no suitable formulation and no paediatric red flag route. ● Known penicillin or beta-lactam allergy, or any history of cephalosporin allergy. Use the metronidazole arm. ● Known significant renal impairment. Refer. A short bridging course is

	not the place for a dose adjustment. ● Infectious mononucleosis or acute lymphoblastic leukaemia, because of the risk of a widespread rash. ● Already taking an antibiotic, for this or any other indication. ● A course already supplied for this episode. One supply per episode. ● Unable to give valid informed consent.
Cautions	**PREGNANCY: amoxicillin may be supplied. It is well established in pregnancy and is the usual choice where an antibiotic is indicated.** **BREASTFEEDING: amoxicillin may be supplied. Small amounts appear in breast milk; this is not a reason to withhold it or to interrupt feeding.** Advise the patient to complete the course even if the pain settles, and that finishing it does not remove the need for the dental appointment. Diarrhoea is common. Advise the patient to seek advice for severe or bloody diarrhoea, which may indicate Clostridioides difficile infection. Oral thrush can follow a course of amoxicillin. Mention it, and say it is treatable. **This PGD supplies no analgesia. Where the patient needs pain relief, sell it as a pharmacy medicine under the pharmacy own protocol and record that you did.**
Actions if the patient is excluded or declines	● **Where the infection is localised and no antibiotic is indicated, say so plainly and explain why: antibiotics do not treat a localised dental infection, the abscess is largely walled off from the bloodstream so very little antibiotic reaches it, and the treatment is drainage or removal of the cause by a dentist. Offer analgesia advice and help the patient get a dental appointment.** ● **Where an emergency red flag is present, make the urgency explicit and act on it in front of the patient. Do not send them away with an appointment to make.** ● Where the exclusion is renal impairment, infectious mononucleosis or an antibiotic already being taken, say plainly that the issue is the antibiotic and not the dental problem, so the patient does not leave thinking their infection has been dismissed. ● Document the reason for exclusion, the advice given and the decision reached. Inform or refer to the GP or dentist as appropriate.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	● That valid informed consent was given. ● **WHICH of the three outcomes applied, and the finding that decided it. Where an antibiotic was supplied, the specific sign of spreading or systemic infection, or the higher-risk factor, that justified it.** ● **That Appendix 1 was worked through and no emergency red flag was present.** ● Temperature, and the presence or absence of facial swelling and lymphadenopathy. ● The penicillin allergy history in the patient own terms, and which arm was used and why. ● That renal function was asked about and no significant impairment was reported. ● That the patient was told this is a bridge and not a treatment, and that a dental appointment is still needed. ● Patient name, address, date of birth, and the GP with whom they are registered.

	• Name and registration number of the healthcare professional supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied, with batch number and expiry date. • Advice given, including advice given if excluded or declining. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Retain for 8 years.

The medicine

Name, form and strength	Amoxicillin 500mg capsules.
Legal category	POM.
Route and method	Oral. Swallow whole with water. May be taken with or without food.
Dose and frequency	500mg three times daily, one capsule every 8 hours, for 5 days.
Quantity to be supplied	15 capsules. Supply the whole course; do not split it.
Maximum treatment period	5 days. One course per episode. A second course is not authorised under this PGD; refer.
Storage	Store below 25C in the original container. Do not supply after the expiry date.
Adverse effects	Common: diarrhoea, nausea, rash (a non-allergic maculopapular rash is common and is not the same as allergy). Uncommon: vomiting, abdominal discomfort, oral candidiasis. Rare: allergic reactions with rash, urticaria or pruritus. Very rare: anaphylaxis, Stevens-Johnson syndrome, hepatitis and cholestatic jaundice, Clostridioides difficile colitis. Consult the current SPC.

Information for the patient

Written information	Supply the amoxicillin patient information leaflet. Do not supply an ibuprofen leaflet: no analgesia is supplied under this PGD.
Counselling	• **THIS IS A BRIDGE, NOT A CURE. The antibiotic will slow the infection down. It cannot drain the abscess or fix the tooth. You still need to see a dentist urgently, within 24 to 48 hours.** • Finish the whole course even if the pain settles. • Take one capsule every 8 hours, with or without food. • **Come back or seek urgent help the same day if the swelling spreads, your eye starts to close, you cannot open your mouth properly, you have difficulty swallowing or breathing, or you feel much worse. Call 999 for breathing or swallowing difficulty.** • Some diarrhoea is common. Get advice if it is severe or bloody. • A rash that appears with this antibiotic is usually not an allergy, but get it checked, and seek urgent help for any swelling of the lips or tongue or any wheeze. • For pain relief, ask us: we can sell you something suitable over the counter.
Follow-up	Arrange the dental appointment before the patient leaves where possible, or give them the NHS 111 route to emergency dental care. Advise that pain should improve within 24 to 48 hours, and that worsening pain, worsening fever or spreading swelling needs the same-day routes above rather than a

	wait.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- Scottish Dental Clinical Effectiveness Programme, Drug Prescribing for Dentistry, dental abscess; NICE Clinical Knowledge Summaries, Dental abscess; NHS primary care antimicrobial prescribing guidance. Summarised 9 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v010
Supersedes	Version 009, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
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Valid from date	30 September 2026
Expiry date	31 July 2027

Change history to version 006

Later versions are recorded in the Version and change record at the end of this document.

Version	v006
Date	10 September 2026
Changes	● RIGORS REMOVED FROM THE SIGNS THAT QUALIFY A PATIENT FOR A SUPPLY. Version 005 listed rigors as a sign of systemic involvement in the inclusion criteria of both arms and in Appendix 2, and also as a sepsis red flag in Appendix 1 that refers to 999 or same-day care. The same finding included and excluded the same patient, on the presentation where delay matters most. Rigors are now a red flag only. ● SPREAD TO THE NECK REMOVED FROM THE INCLUSION CRITERIA. Version 005 counted cellulitis spreading into the soft tissues of the face or neck as a reason to supply, while the cover and Appendix 1 treated swelling extending down the neck as an airway emergency. Neck involvement is now an emergency only. Both found by the clinical review of 10 September 2026.

Previous versions

005, 9 September 2026	Guidance summary restored as part 2; indication restored the right way round (spreading or systemic infection qualifies, localised does not); three outcomes stated once; higher-risk route for immunosuppression and poorly controlled diabetes; Appendix 2 added; pregnancy and breastfeeding back as cautions in the amoxicillin arm; practitioner pages and standard training requirements restored; SDCEP versus CKS divergence on the penicillin-allergy arm recorded; amoxicillin course 5 days.
004, 9 September 2026	See the change history of that version. Superseded the same day by v005, which restores the guidance summary section.
003, 8 September 2026	Metronidazole arm added for penicillin-allergic adults at the licensed 200mg dose; alcohol handled as an inclusion criterion; interaction exclusions and Cockayne syndrome added; red flags collected into an appendix. The indication was inverted in this version and is corrected in 004.
002, 7 September 2026	Full clinical review following feedback from an adopting pharmacy. The ibuprofen arm was removed, ibuprofen being a P medicine for which a PGD is not required. Amoxicillin only, adults 18 and over.
001, earlier 2026	Development and issue of new PGD. Amoxicillin and ibuprofen arms.

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Patient Group Direction

for the supply, for the treatment of Acute Dental Infection, as a bridging antibiotic, of:

Metronidazole, penicillin allergy

Patient Group Direction for the supply of metronidazole 200mg tablets as a bridging antibiotic in acute dental infection with spreading or systemic involvement, in penicillin-allergic adults aged 18 years and over.

The licensed dose for acute dental infection is 200mg three times daily. SDCEP suggests 400mg 8-hourly, which is above the licensed dose for this indication. This PGD stays within the licence.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- Must have completed training relevant to acute dental infection, documented and overseen by the Get Real Health team.
- Must be able to distinguish a LOCALISED dental infection from a SPREADING or SYSTEMIC one, because that distinction decides whether an antibiotic is indicated at all. This is the single most important competence for this PGD.
- Must be able to recognise every emergency red flag in Appendix 1, in particular airway compromise, difficulty swallowing, trismus and periorbital involvement, and must act on them the same day rather than supplying.
- Must be able to explain to a patient with a localised infection why an antibiotic is not being supplied, and why that is the right answer rather than a refusal of care.
- Must be able to take and record a penicillin allergy history in the patient own words, distinguishing true allergy from intolerance.
- Must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005. This PGD covers adults only.

The PGD

Indication	Bridging antibiotic care for an acute dental infection with signs of spreading infection or systemic involvement, in a penicillin-allergic adult awaiting urgent dental assessment. This is not definitive treatment.
The three outcomes	**Decide which of these applies BEFORE considering any medicine.**

	1. LOCALISED INFECTION ONLY, otherwise healthy patient: NO ANTIBIOTIC. Analgesia advice and an urgent dental appointment. This is most patients who present, and it is the correct answer for them. **2. SPREADING OR SYSTEMIC INFECTION, no emergency red flag: ANTIBIOTIC UNDER THIS PGD, and an urgent dental appointment as well. This is the bridging cohort.** **3. ANY EMERGENCY RED FLAG from Appendix 1: 999 or same-day emergency care. No antibiotic here, and no delay to arrange one.** **An antibiotic under this PGD is a bridge to dental care, not a treatment. It does not drain the infection and it does not remove the cause. The dental appointment is the treatment, and it is needed whether or not an antibiotic is supplied.**
Inclusion criteria	● Adults aged 18 years and over. ● An acute dental infection with a presumed bacterial source in a tooth or its supporting tissues. ● **AT LEAST ONE sign of spreading infection or systemic involvement, from the list below.** ● Temperature 38C or above. ● Regional lymphadenopathy: tender, enlarged nodes in the neck or under the jaw. ● Facial swelling, or swelling extending beyond the tooth and its immediate gum, that does NOT meet any emergency red flag in Appendix 1. ● Malaise, feeling generally unwell. RIGORS ARE NOT ON THIS LIST: a rigor is a sepsis red flag in Appendix 1 and refers. ● Cellulitis: diffuse redness and swelling spreading into the soft tissues of the face, without red flags. ANY SPREAD TO THE NECK is an Appendix 1 emergency, not a reason to supply. ● **OR the patient is at higher risk of complications from a dental infection even where it appears localised: significant immunosuppression, or poorly controlled diabetes. Record which applies and why you judged the risk higher.** ● **No emergency red flag from Appendix 1 present.** ● Unable to obtain definitive dental treatment before the infection would be expected to worsen, and willing and able to arrange an urgent dental appointment within 24 to 48 hours. ● **Penicillin-allergic, including beta-lactam or cephalosporin allergy, so the amoxicillin arm cannot be used.** ● Capable of giving valid informed consent, and consent given. ● No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following is present.** ● **ANY emergency red flag from Appendix 1. These are 999 or same-day emergency presentations, not dental appointments.** ● **A LOCALISED infection with no sign of spread and no systemic feature, in a patient who is not at higher risk. An antibiotic is not indicated. Give analgesia advice and arrange urgent dental care. This is not a failure of the service; it is the service working correctly.** ● Under 18 years of age. Children with dental infection are referred: this PGD carries no paediatric dose, no suitable formulation and no paediatric red flag route. ● **Pregnant or breastfeeding. The SPC advises metronidazole should not be given in pregnancy or lactation unless considered essential, which is a prescriber judgement and not a PGD one.**

	Refer. ● **Unable or unwilling to avoid alcohol completely during the course and for 48 hours afterwards. Ask directly and record the answer.** ● Taking warfarin or another coumarin, lithium, disulfiram, busulfan, 5-fluorouracil, ciclosporin, phenytoin, phenobarbital, or a QT-prolonging medicine. ● Cockayne syndrome. Absolute exclusion, following reports of severe and sometimes fatal hepatotoxicity of very rapid onset. ● Known hypersensitivity to metronidazole or other nitroimidazoles. ● Severe hepatic impairment, or active neurological disease. ● Already taking an antibiotic, for this or any other indication. ● A course already supplied for this episode. One supply per episode. ● Unable to give valid informed consent.
Cautions	**Alcohol is asked about before the decision to supply is made, and inability or unwillingness to avoid it is an exclusion in this arm (above), not a counselling point, because the reaction can be severe.** A metallic taste and furred tongue are common and settle after the course. Advise the patient to seek advice for numbness or tingling in the hands or feet, which can indicate peripheral neuropathy. Advise the patient to complete the course, and that finishing it does not remove the need for the dental appointment. This PGD supplies no analgesia. Where the patient needs pain relief, sell it as a pharmacy medicine under the pharmacy own protocol and record that you did.
Actions if the patient is excluded or declines	● **Where the infection is localised and no antibiotic is indicated, say so plainly and explain why: antibiotics do not treat a localised dental infection, the abscess is largely walled off from the bloodstream so very little antibiotic reaches it, and the treatment is drainage or removal of the cause by a dentist. Offer analgesia advice and help the patient get a dental appointment.** ● **Where an emergency red flag is present, make the urgency explicit and act on it in front of the patient. Do not send them away with an appointment to make.** ● Where the exclusion is an interacting medicine, alcohol or pregnancy, say plainly that the issue is the antibiotic and not the dental problem, so the patient does not leave thinking their infection has been dismissed. ● Document the reason for exclusion, the advice given and the decision reached. Inform or refer to the GP or dentist as appropriate.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	● That valid informed consent was given. ● **WHICH of the three outcomes applied, and the finding that decided it. Where an antibiotic was supplied, the specific sign of spreading or systemic infection, or the higher-risk factor, that justified it.** ● **That Appendix 1 was worked through and no emergency red flag was present.** ● Temperature, and the presence or absence of facial swelling and lymphadenopathy. ● The penicillin allergy history in the patient own terms, and which arm was used and why.

	• **That the alcohol rule was explained and the patient confirmed they can keep to it.** • That the patient was told this is a bridge and not a treatment, and that a dental appointment is still needed. • Patient name, address, date of birth, and the GP with whom they are registered. • Name and registration number of the healthcare professional supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied, with batch number and expiry date. • Advice given, including advice given if excluded or declining. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Retain for 8 years.

The medicine

Name, form and strength	Metronidazole 200mg tablets.
Legal category	POM.
Route and method	Oral. Swallow with water, with or after food to reduce nausea.
Dose and frequency	200mg three times daily for 5 days. This is the licensed dose for acute dental infection; the SPC gives 200mg three times daily for 3 to 7 days.
Quantity to be supplied	15 tablets. Supply the whole course; do not split it.
Maximum treatment period	5 days. One course per episode. A second course is not authorised under this PGD; refer.
Storage	Store below 25C in the original container, protected from light. Do not supply after the expiry date.
Adverse effects	Common: metallic taste, nausea, furred tongue, gastrointestinal upset. Uncommon: headache, dizziness, dry mouth. Rare: peripheral neuropathy with prolonged use, darkened urine, hypersensitivity reactions. Very rare: encephalopathy, seizures, hepatotoxicity, and severe hepatotoxicity of rapid onset in Cockayne syndrome. Consult the current SPC.

Information for the patient

Written information	Supply the metronidazole patient information leaflet. Do not supply an ibuprofen leaflet: no analgesia is supplied under this PGD.
Counselling	• **NO ALCOHOL AT ALL during the course and for 48 hours after the last tablet. That includes wine, beer, spirits, and alcohol in medicines such as some cough remedies and mouthwashes. The reaction causes flushing, vomiting and a racing heart.** • **THIS IS A BRIDGE, NOT A CURE. You still need an urgent dental appointment within 24 to 48 hours.** • Finish the whole course. Take one tablet every 8 hours, with or after food. • A metallic taste is common and goes when the course finishes. • **Seek urgent help the same day if the swelling spreads, your eye starts to close, you cannot open your mouth properly, or you have difficulty swallowing or breathing. Call 999 for breathing or**

	swallowing difficulty. ● Tell us if you get numbness or pins and needles in your hands or feet. ● For pain relief, ask us: we can sell you something suitable over the counter.
Follow-up	As the amoxicillin arm. Arrange the dental appointment before the patient leaves where possible, or give the NHS 111 route to emergency dental care.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- Scottish Dental Clinical Effectiveness Programme, Drug Prescribing for Dentistry, dental abscess; NICE Clinical Knowledge Summaries, Dental abscess; NHS primary care antimicrobial prescribing guidance. Summarised 9 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v010
Supersedes	Version 009, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
Valid from date	30 September 2026
Expiry date	31 July 2027

Change history to version 006

Later versions are recorded in the Version and change record at the end of this document.

Version	v006
Date	10 September 2026
Changes	● RIGORS REMOVED FROM THE SIGNS THAT QUALIFY A PATIENT FOR A SUPPLY. Version 005 listed rigors as a sign of systemic involvement in the inclusion criteria of both arms and in Appendix 2, and also as a sepsis red flag in Appendix 1 that refers to 999 or same-day care. The same finding included and excluded the same patient, on the presentation where delay matters most. Rigors are now a red flag only. ● SPREAD TO THE NECK REMOVED FROM THE INCLUSION CRITERIA. Version 005 counted cellulitis spreading into the soft tissues of the face or neck as a reason to supply, while the cover and Appendix 1 treated swelling extending down the neck as an airway emergency. Neck involvement is now an emergency only. Both found by the clinical review of 10 September 2026.

Previous versions

005, 9 September 2026	Guidance summary restored as part 2; indication restored the right way round (spreading or systemic infection qualifies, localised does not); three outcomes stated once; higher-risk route for immunosuppression and poorly controlled diabetes; Appendix 2 added; pregnancy and breastfeeding back as cautions in the amoxicillin arm; practitioner pages and standard training requirements restored; SDCEP versus CKS divergence on the penicillin-allergy arm recorded; amoxicillin course 5 days.
004, 9 September 2026	See the change history of that version. Superseded the same day by v005, which restores the guidance summary section.
003, 8 September 2026	Metronidazole arm added for penicillin-allergic adults at the licensed 200mg dose; alcohol handled as an inclusion criterion; interaction exclusions and Cockayne syndrome added; red flags collected into an appendix. The indication was inverted in this version and is corrected in 004.
002, 7 September 2026	Full clinical review following feedback from an adopting pharmacy. The ibuprofen arm was removed, ibuprofen being a P medicine for which a PGD is not required. Amoxicillin only, adults 18 and over.
001, earlier 2026	Development and issue of new PGD. Amoxicillin and ibuprofen arms.

Appendix 1: Emergency red flags. These are 999 or same-day emergency care, not an antibiotic and not a dental appointment.

If any of these is present, act on it now. Do not supply an antibiotic first, and do not let arranging one delay the referral.

- **Difficulty breathing, or any change in the voice. Call 999.**
- **Difficulty swallowing, or drooling because the patient cannot swallow saliva. Call 999.**
- **Swelling of the floor of the mouth, or a raised or displaced tongue. This suggests Ludwig angina. Call 999.**
- **Trismus: the patient cannot open the mouth more than about two finger widths. Same-day emergency assessment.**
- **Periorbital or orbital involvement: swelling closing the eye, eye pain, double vision or reduced vision. Same-day emergency assessment.**
- Rapidly spreading swelling, or swelling extending down the neck.
- Signs of sepsis: rigors, confusion, very rapid breathing or heart rate, mottled or ashen skin, not passing urine.
- Systemically very unwell in a way that does not fit a simple dental infection, however the individual observations read.

Appendix 2: Deciding localised versus spreading

LOCALISED, so no antibiotic:

- Pain and tenderness at one tooth, with or without a small amount of gum swelling immediately next to it.
- Purulent discharge from the gum next to the tooth, without spread.
- No fever, no lymphadenopathy, no facial swelling, patient otherwise well.

SPREADING OR SYSTEMIC, so an antibiotic is indicated as a bridge:

- Temperature 38C or above.
- Tender enlarged nodes in the neck or under the jaw.
- Facial swelling, or swelling beyond the immediate gum, without any Appendix 1 red flag.
- Diffuse redness and swelling spreading into the soft tissues of the face, not the neck.
- Malaise, feeling generally unwell.
- **NOT rigors and NOT any spread to the neck: both are Appendix 1 emergencies and go to 999 or same-day care, never to a supply.**

If you cannot decide which of the two applies, treat the patient as needing assessment rather than a supply, and say so.

Appendix 3: Key references

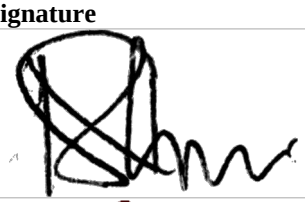
- Scottish Dental Clinical Effectiveness Programme, Drug Prescribing for Dentistry, dental abscess, current edition.
- NICE Clinical Knowledge Summaries, Dental abscess, current revision.
- NHS primary care antimicrobial prescribing guidance for dental infection, current local formulary.
- Summary of Product Characteristics for amoxicillin 500mg capsules and metronidazole 200mg tablets, current versions.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Version and change record
Version: v010
Issued: 30 September 2026
Supersedes: Version 009, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v009
Issued: 14 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Amoxicillin arm 'actions if excluded' no longer refers to alcohol or pregnancy, which are metronidazole arm exclusions; it names the amoxicillin arm's own exclusions.
Training bullet no longer refers to consent in children and young people in an adult-only PGD.
Metronidazole arm inclusion now names beta-lactam and cephalosporin allergy alongside penicillin allergy, matching the amoxicillin arm exclusion.
Metronidazole arm caution now says alcohol is an exclusion, matching the exclusion list (it said inclusion criterion).
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v010, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.